جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : انصاف السيد ابوالمجد حسنين
المحافظة : القاهره **الحي / المدينة :** الساحل **العنوان :** -
رقم البطاقة : 24804010102246 **رقم التليفون :** 01211389893 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 01-04-1948 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: انصاف السيد ابوالمجد حسنين
National ID: 24804010102246

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف روسي
انترناشونال مصر

جوستاف روسي

السيد أ.د: مدير عام المجالس الطبية بوزارة
تحيه طبية وبعد

كشف حساب

بيان بنفقات المريض/ آنهاف السيد علاج على نفقة الدوله

عن الفترة من ١٤/٧/٢٠٢٠ الى ١١/٤/٢٠٢٠

البيان	رصيد القرار	قيمة المنصرف
رصيد القرار رقم	٣٠٠٠	١١١٤,٨٦
اقامه		
كشف		
اشراف طبى		
اشعـــــه		
رنين- مقطعيه - اخرى		
تحاليل طبيه		
مستلزمات		
علاجات متنوعه		
بنك دم – علاج		
الاندوبه		
الاجمـــــــالـــى		١١١٤,٨٦

GRI-E
جوستاف روسي
جوستاف روسي

المراجع

المرظف المختص
رنا

Non-HIO Letter
[[DOCTYPE:NON_HIO_LETTER]]



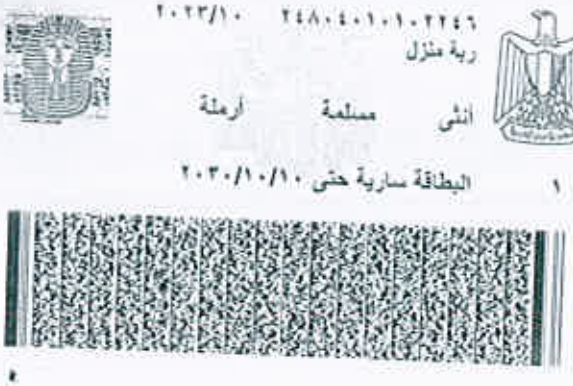
National ID – Front
[[DOCTYPE:ID_FRONT]]

٢٤٨٠٤٠١٠١٠٢٢٤٦ ٢٠٢٣/١٠

ربة منزل

انثى مسلمة أرملة

البطاقة سارية حتى ٢٠٣٠/١٠/١٠



٢/٤

National ID – Back
[[DOCTYPE:ID_BACK]]

Patient Name	: الأستاذه / انصاف السيد ابوالمجد حسنين	Request No.	: 13008359
Patient No.	: 24804010102246	Request Date	: 01/02/2025
Path No.	: 421/25	Verification Date	: 16/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Her2: Negative (score 0).
- Ki67: Positive in 75% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / انصاف السيد ابو المجد حسنين	Request No.	: 13008359
Patient No.	: 24804010102246	Request Date	: 01/02/2025
Path No.	: 421/25	Verification Date	: 16/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast malignant mass.

Nature of Specimen: Tru-cut biopsy, collected at 30/1/2025.

Gross Examination:
- Four grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Angioinvasion: No.
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 9% of tumor cells (score 5 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 75% nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8 /8).
- PR: Positive (score 5 /8).

Verified by :
DR.Menna Nabil

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ROUSSY
INTERNATIONAL
EGYPT

Name: Mrs. Ansaf El sayed Aboel magd
Date: 24/11/2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: pathologically proven breast cancer , received NACT

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Deep irregular speculated high density mass lesions with surrounding distortion.
- Thickened skin and coarsened trabeculae.
- **Ultrasound** demonstrated irregular spiculated heterogenous hypoechoic solid mass lesions with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study seen at:
  - Index Lesion: 6 o'clock, 3 cm from nipple, deeply seated inseparable from underlying muscle layer , measuring 27 x 15 mm. Two satellite lesions are noted (1.2 cm apart) measuring 1.2 x 0.5 and 0.5 x 0.5 cm .
  - Another lesion of similar characters is noted at right axillary tail measuring 1.3 x 1 cm .
- Thickened skin reaching 5 mm and coarsened trabeculae.
- No suspicious axillary lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated No solid or cystic lesions
- Retro-areolar dilated ducts reaching 4 mm with clear content/
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Multicentric locally advanced pathological process as described .

Left breast (BIRADS 2): Retro-areolar duct ectasia with clear content .

Best Regards,

DR. Israa Reda , M.Sc.

For Comparison with last mammogram

Cr. 6/1c/12

- Mild regressive course of widet mass lesion noted at 6o'clock measures 27x15 mm (was 35x35 mm) w/ two adjacent Satellites noted measures 1.2x0.5 cm and 0.5x0.5 cm.
- Stationary axillary tail lesion (? LN w extra capsular invasion) measures 1.1x1 cm (not more than 1 mm).

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL
KOPIE

Patient Name: Ensaaf Al Sayed Aboelmagd Hasanain	Date : 22/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 77 Year(s)
Medical ID: 313/2025	Pat. ID: 97242

MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS FOLLOWING ORAL AND INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:

Clinical data: *right breast cancer on hormonal therapy as compared with previous CT dated 8/3/2025 the current study*

- *Focal spiculated right breast mass it measures currently 2.7x2 cm*
- *Still noted Multiple bilobar hypodense discrete focal hepatic lesions are seen , the largest is measures about 3.3x3.2 cm with very faint / subtle contrast enhancement and continuous contrast poor enhancement in the delayed phase to become more isodense to the liver parenchyma….features are suggestive of multiple focal hepatic deposits (stationary course)*

-lower lumbar decompression laminectomy with fixation by rods and screws and no suspicious bony lesions

Otherwise;

- *Normal lung parenchyma, showing no focal lesions.*
- *Normal vascular mediastinal structures.*
- *The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.*
- *No evidence of mediastinal or hilar nodal enlargement .*
- *No pleural collections.*
- *The intra and extra-hepatic biliary radicles are normal.*
- *The pancreas and spleen are normal.*
- *Normal CT features of both kidneys.*
- *Right and left supra renal glands are normal.*
- *No CT evidence of enlarged abdominal or pelvic lymph nodes.*
- *No ascites.*
- *Scans through the pelvis showed no masses.*

Much Obliged;
Dr.Khaled Moghazy,MD
Prof.of Radiology

CT Scan
[[DOCTYPE:CT_SCAN]]

MDT form

A-Patient ID:
1- Name :
Sex
3- Medical number: 83257815
Female
Old Mec :
313-2025
2-ID number:
Patient Age : 77 Y



Family history: +ve
Phone: 01211389893
BMI:
REASON OF MDT:
marital status:
NRS Score:
GROUP:
Date of MDT: 17/02/2025
Co morbidity: htn,dm, neurological fixation
Menopausal status:
Initial STAGE:

Clinical data

Breast Imaging :
28/1/2025 rt breast mass 6 o clock 3.5 cm reaching skin
multiple rt axillary LN level I extra capsular invasion 1.3 cm

Mets Workup :
cap: LIVER METS
bone scan:L1 increased uptake

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC GII	8/8	5/8	0		75

T	N	M
CT4C	1	1 LIVER

Sonomammography

PATIENT HISTORY
started 1 yr ago with rt breast lump

MDT OUTCOME & PLAN

FOR FIRST LINE AROMATASE INHIBITOR

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	الأستاذه / انصاف السيد ابو المجد حسنين :	Request No.	: 13008359
Patient No.	: 24804010102246	Request Date	: 01/02/2025
Path No.	: 421/25	Verification Date	: 16/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

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- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 75% nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8 /8).
- PR: Positive (score 5 /8).

Verified by :
DR.Menna Nabil

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / انصاف السيد ابوالمجد حسنين	Request No.	: 13008359
Patient No.	: 24804010102246	Request Date	: 01/02/2025
Path No.	: 421/25	Verification Date	: 16/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Ensaaf Al Sayed Aboelmagd Hasanain	Date : 13/02/2025
Exam : C.T CHEST	Age : 77 Year(s)
Medical ID: 313/2025	Pat. ID: 97242

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast LQ speculated mass lesion, measuring 3.3 X 2.9 cm. associated with mild skin thickening
- Right axillary level I and II pathologically enlarged lymph nodes, the largest measures 1.8 x 1.7 cm.
- Few left axillary level I prominent lymph nodes, the largest measures about 1.7x0.8 cm.
- Few subcentimetric right supra-clavicular lymph nodes.
- Few bilateral scattered pulmonary nodules, the largest is seen fissural based at anterior6 segment of left lower lung lobe measuring 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal non enhanced CT appearance of the thoracic aorta and the great vessels apart from calcifications.

2. Abdomen and pelvis:

- The liver is average in size with **heterogenous CT texture. Showing bilobar hypodense discrete and confluent focal lesions, the largest is seen at segment II/IVa and measures about 5.5x2.7 cm on non-contrast basis.**
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with marginal calcifications are noted with no focal lesions on non-contrast basis.
- Bilateral few subcentemetric para-aortic, para caval and mesenteric lymph nodes.
- Area of misty / smudged mesenteric fat planes are noted with slight whorly appearance, seen centrally inferior to the pancreas.
- Normal size, parenchymal thickness & density of both kidneys (left renal gravels) with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Underfilling of the urinary bladder, however no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Ensaaf Al Sayed Aboelmagd Hasanain	Date : 13/02/2025
Exam : C.T CHEST	Age : 77 Year(s)
Medical ID: 313/2025	Pat. ID: 97242

3. The scanned skeleton:

- Left femoral neck small sclerotic lesion ? Bone island, for bone scan correlation if clinically warrantied.
- ***Otherwise***, The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.
- Marked spondylodegenerative changes.
- L3 to S1 internal fixation with plate and screws.

OPINION:

- ***Right breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.***
- ***Bilateral scattered pulmonary nodules likely for follow up.***
- ***Hepatic bilobar focal lesions likely metastatic for triphasic study.***

Dr.Maie Khattab, MSc.
Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Ensaaf Al Sayed Aboelmagd Hasanain	Date : 28/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 77 Year(s)
Medical ID: 24804010102246	Pat. ID: 97242

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Right breast palpable mass

Report:

- ACR b: scattered fibroglandular parenchyma
- Right Breast:
  - Rounded spiculated dense mass is seen in the right lower central region (around 6 O'clock) in the inframammary fold. By ultrasound it appears as an invasive hypoechoic mass measuring about 35x35mm reaching the skin 9mm from the nipple and resting on the chest wall.
  - No overlying microcalcification.
  - Multiple (>3) pathologically enlarged level I axillary lymph nodes are noted showing thick cortices and effaced hila. The largest shows irregular outline denoting extracapsular invasion measuring 13mm in diameters.
  - No supra or infraclavicular lymphadenopathy.
- Left Breast:
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Right breast malignant mass and level I axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of left breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
28/01/2025 4:40 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name : ANSAF ELSAYED ABO ELMAGD
Study Date : 03 / 02 / 2025
Dear Prof.DR :

MRI OF THE LUMBOSACRAL SPINE

TECHNIQUE:

- Axial and Sagittal T1 and T2 WIs
- Post contrast T1 WIs

FINDINGS:



- Evidence of spinolaminectomy of L4 and L5 with decompression of the thecal sac and internal fixation of L4, L5 and S1 by trans pedicular screws and two rods causing ferromagnetic artifact.
- Mild forward slippage of L4 over L5.
- Lumbar spondylosis evident by marginal osteophytic lipping and degenerative marrow signal of the vertebral endplates and degeneration of the lumbar discs.
- L1-2 posterior and right posterolateral disc protrusion is seen indenting the theca and encroaching upon the both neural exit foramina more to the right side.
- L2-3 and lesser extent L3-4 diffuse posterior disc bulges are seen indenting the theca and encroaching upon corresponding neural foramina.
- L4-5 central disc protrusion with cranial migration is seen indenting the theca with partial encroachment upon the neural foramina.
- Low T1&T2 marrow signal seen related to D12/L1 vertebral end plates with no appreciable post contrast enhancement, likely sclerotic changes, for further bone scan correlation.
- Normal size and signal of the conus medullaris.
- No para-spinal soft tissue masses or collection.

OPINION:

- **Post-operative study showing the above described changes.**
- **Kindly correlate with the previous unavailable examination.**

Much Obliged,
Dr.Mahmoud ELGamal, FEBR